

Site of Care and Drug Benefit Alignment Program Management Document

Client: **UPS**
Medical Carrier: **Cigna**
Program Effective Date: **1/1/2019**

INTRODUCTION

UPS has elected to implement the Site of Care and Drug Benefit Alignment program through CVS Caremark®. This document provides an overview of the program requirements and processes that will be managed Cigna and CVS Caremark to support **UPS** and their plan members. It is a summary of the information discussed during implementation to support post-implementation activities for the program. It may be used as a reference or resource guide for **UPS, Cigna** and any other entity that participates in the active management and oversight of members impacted by this program.

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BACKGROUND/OVERVIEW

Effective **1/1/2019**, select specialty medications that were previously covered under the UPS medical plan will be covered exclusively under UPS's prescription drug benefit administered by CVS Caremark (exceptions noted below). UPS has selected CVS Specialty® as their exclusive provider for these specialty medications (refer to the UPS Site of Care & Drug Benefit Alignment drug list). The following changes listed under each program component are being implemented.

- **Site of Care (Infused Medications)**
 - Members receiving these infused medications must receive these medications through CVS Specialty.
 - Exceptions: Inpatient hospital and Emergency Room claims will continue to be payable through Cigna.
 - The CVS Specialty CareTeam will work with the provider to discuss alternate, in-network lower cost site of care options such as home care, ambulatory infusion centers or the provider's office (where allowed by law).
- **Drug Benefit Alignment (Self-Administered and Clinician-Administered Medications)**
 - For members receiving **self-administered medications**, CVS Specialty will work directly with members to dispense the medication to a location of their choice (where allowed by law).
 - For members receiving **clinician-administered medications**, CVS Specialty will work with members and their providers to arrange convenient delivery to the provider office (where allowed by law) or to the member home for administration by a home health nurse.
 - Exceptions: Inpatient hospital and Emergency Room claims will continue to be payable through Cigna.

Program Scope	Effective 1/1/19, select specialty drugs meeting the following criteria will not be payable through Cigna and must be dispensed through CVS Specialty under the pharmacy benefit. Program Inclusion Criteria: • Drugs: All drugs on the UPS Site of Care & Drug Benefit Alignment Drug List • Place of Service: Applies to all places of service except inpatient hospital (CMS POS code 21) and emergency room (CMS POS code 23) Program Exclusions: The following are excluded from the program and may continue to be payable through Cigna. • Medicare primary members • Drugs billed by facility as part of an inpatient hospital stay or emergency room visit
Prior Authorization and Eligibility Process	• Cigna will redirect all prior authorization (PA) and eligibility requests meeting the criteria above to CVS Caremark's dedicated line for UPS at 1-855-282-8412.
Claims Process	Effective 1/1/19: • Cigna will deny any professional and facility claims associated with all <i>active</i> Healthcare Common Procedure Coding System (HCPCS) codes on the UPS Site of Care & Drug Benefit Alignment Drug List except for the following exclusions: ▪ Inpatient hospital (CMS POS code 21) and emergency room (CMS POS code 23) claims ▪ Claims where Medicare is the primary carrier • While Cigna's claims system cannot deny facility claims at the HCPCS level, it will deny due to the lack of meeting the authorization requirement for payment of facility claims (see Prior Authorization and Eligibility Process). • The following explanation of benefit (EOB) message will be applied to the denied claims: ▪ Member EOB Description: This specialty medication is not covered under the medical plan. This expense may be covered under the prescription plan. Please contact CVS Caremark at 1-855-282-8412 for more information. – Provider EOB Description: Same as above
Grace Fill Process	Effective 1/1/19: • Each member will be allowed one grace fill (a one-time payment under the medical benefit) at the drug level for any specialty medication that meets the program inclusion criteria above at UPS's discretion. The one-time payment may include multiple dates of service. • UPS will provide the approval to Cigna to pay the grace fill per the following process: – If a member with a denied claim escalates to UPS, UPS will forward the member information to the CVS Specialty SPOC team: SRX-BISPOCAccount@CVSCaremark.com – In addition, Cigna will provide a quarterly file identifying any members with paid medical claims to CVS Specialty (See Medical Utilization Review section below). – CVS Specialty will provide telephonic outreach and education to the member and provider, advising them the drug must be obtained through CVS Specialty under the pharmacy benefit. – Once the transition has been completed, CVS will notify UPS Amy Nations.

	- UPS will then notify the following Cigna contacts to approve payment of past medical claims: - Trina Bailey trina.bailey@cigna.com - Rachel Fowler Rachel.Fowler@Cigna.com
Waiver (Benefit Exception) Process **Applies to Site of Care Medications only**	- CVS Specialty SPOC team will notify UPS contact, Amy Nations, of any exception requests made by a member and/or provider - UPS will review and approve/deny requests. Pediatric members or drugs infused in conjunction with oncology medications may qualify for an exception, provided UPS approves. - UPS will facilitate benefit exceptions by providing the approval directly to Cigna to allow claims to pay.

Maintenance/Ongoing Processes *(Supported by CVS Caremark Account Management Team)*

Drug List Updates Process	- The drug list updates will be available on a quarterly basis in January, April, July and October. - The CVS Caremark Account Team will send drug list updates one to two weeks prior to the beginning of each quarter to: - Rachel Fowler, Acct Mgr - Rachel.Fowler@Cigna.com - Trina Bailey, Client Service Executive - trina.bailey@cigna.com - Rhonda Guffey, Implementation Manager Rhonda.Guffey@Cigna.com - Amy Nations - anations@UPS.com - The first update will include Q1 and Q2 2019 updates and will be provided one to two weeks before April 1, 2019. - Cigna will update the drug list for all applicable service areas and for their claims system coding within 30 business days. The effective date will coincide with when the updates go live in the claims system to avoid retroactive reprocessing. - Cigna will send confirmation email to advise when system coding is complete to: - Sydney Matejka - Sydney.Matejka@CVSHealth.com - Lee Trevigne - Lee.Trevigne@CVSHealth.com - Tierra Ford - Tierra.Ford@CVSHealth.com - Amy Nations - anations@UPS.com
Medical Utilization Review	Starting April, 2019, Cigna will generate a quarterly report to identify any claims paid under medical for drugs on the most current UPS Site of Care & Drug Benefit Alignment Drug List. This report will be utilized for member and provider outreach. - The report will include paid professional and facility medical claims with dates of service incurred within the previous six months pertaining to the full list of HCPCS codes (both ACTIVE and HISTORICAL HCPCS) on the most recent drug list. With the exception of the April 2019 report, the paid date range should include the most recent three months and start where the last report ended. <i>Note: The full list of HCPCS codes is located on the FIRST TAB of the drug list.</i> - For example: - The first report in April 2019 will include claims incurred from 1/1/19 through 3/31/19 and paid 1/1/19 through 3/31/19. - The July 2019 report will include claims incurred 1/1/19 through 6/30/19 and paid

4/1/19 through 6/30/19.

- The October 2019 report will include claims incurred 4/1/19 through 9/30/19 and paid 7/1/19 through 9/30/19, and so forth.
- All required data elements (highlighted in green) are found on the "CVS Member Provider Outreach File.xls".
- The report must exclude the following program exclusions:
 - Medicare primary members (will be removed by Wendy Smitherman)
 - Inpatient hospital and emergency room claims
 - Any claims associated with HCPCS not on the UPS drug list.
- **The report will be sent no later than the end of the first month of each quarter (i.e., Jan. 31, April 30, July 31, Oct. 31) via encrypted email (with social security numbers omitted) to:**
 - SRX-BISPOCAccount@CVSCaremark.com
- Cigna will indicate in the email any limitations in meeting the data requirements and confirmation that the program exclusions were removed from the report

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Key Contacts	Cigna	Primary Name, Title: Trina Bailey, Client Service Executive Phone: 860-902-6723 Email: trina.bailey@cigna.com Secondary Name, Title: Rachel Fowler, Account Manager Phone: 404-512-8044 Email: Rachel.Fowler@Cigna.com Name, Title: Rhonda Guffey, Implementation Manager Phone: 423-954-5861 Email: Rhonda.Guffey@Cigna.com
	CVS Caremark Account Team	Primary Name, Title: Sydney Matejka, Director, Strategic Accounts Phone: 847-855-1802 Email: Sydney.Matejka@CVSHealth.com Secondary Name, Title: Lee Trevigne, Sr. Account Manager Phone: 480-614-7725 Email: Lee.Trevigne@CVSHealth.com
	UPS	Name: Amy Nations Phone: 404-828-4383 Email: anations@UPS.com

CVS Specialty Contact Info	Members or providers should be directed to call CVS Caremark's dedicated line for UPS and request to speak with CVS Specialty for new member enrollment: • 1-855-282-8412 • 6 a.m. to 10 p.m. (CT), Monday through Friday
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Frequently Asked Questions (FAQs)

*A reference guide of frequently asked questions and answers for anyone involved in the active management and oversight of members impacted by the **Site of Care & Drug Benefit Alignment program** through CVS Specialty.*

Q1: Are members required to obtain specialty drugs through CVS Specialty?

A1: Yes. Select specialty drugs must be obtained through CVS Specialty under the pharmacy benefit. Providers may contact CVS Caremark at 1-855-282-8412 from 6 a.m. to 10 p.m. (CT), Monday through Friday, and request to speak to CVS Specialty to set up the next fill.

Q2: What specialty drugs are included in this initiative?

A2: Select specialty medications are included in this program and will be solely covered under the pharmacy benefit, including self-administered medications and clinician-administered medications. These drugs may be oral, injectable, inhaled or infused medications. For a full list of medications, members can refer to the benefits administrator or contact CVS Caremark at 1-855-282-8412 and request to speak to CVS Specialty.

Q3: What if a member needs a clinician to administer the medication?

A3: If a member requires nursing assistance or self-administration training, CVS Specialty will arrange nursing services on behalf of the member. Alternatively, the provider may still be able to administer the drug onsite at their clinic or facility. For those situations, CVS Specialty would deliver the medication directly to the doctor's office or facility administering the medication. Any administration charges may still be covered and payable under the member's medical plan administered by Cigna per their provider contract.

Q4: How will CVS Specialty know which site of care is appropriate for an infusion medication?

A4: Many infused specialty medications can be safely administered in patient homes or ambulatory infusion centers. The CVS Specialty CareTeam will work with members receiving these medications and their treating provider(s) to determine the most clinically appropriate and cost-effective administration site for a specific drug.

Q5: What alternate sites of care for infusion drugs are offered through this initiative?

A5: Members who are currently receiving infused medications included on the **UPS Specialty Drug List** may be able to receive the medication at alternate sites of care such as:

- Home infusion: an infusion nurse will administer the drug in the comfort of their own home
- Ambulatory infusion center: a freestanding center with infusion nursing services
- Physician's office

Other members may continue to receive these medications in their current care setting. CVS Specialty will dispense the drug directly to that facility.

Q6: Will this impact members' treatment?

A6: No. The CVS Specialty CareTeam will work with the members' provider(s) to help ensure their needs are met in a clinically appropriate way.

Q7: Who should members or providers call if they have questions about the program?

A7: Members and providers may contact CVS Caremark at 1-855-282-8412 and request to speak to CVS Specialty for more information.

Q8: How do members start filling prescriptions through CVS Specialty?

A8: Members may call CVS Caremark at 1-855-282-8412 and request to speak with CVS Specialty to set up their next fill. The CVS Specialty CareTeam will assist members with the transition, including calling providers, obtaining the prescription, verifying benefits, setting up delivery, etc.

Q9: How can providers start their members with CVS Specialty?

A9: Providers may call CVS Caremark at 1-855-282-8412 and request to speak to CVS Specialty to set up the next fill. They will have to provide a new prescription to transition the dispensing of the specialty drug to CVS Specialty. They may also have to provide clinical information for completion of a prior authorization. For members on select infusion drugs administered in hospital outpatient facilities, the CVS Specialty CareTeam will reach out to the provider(s) to evaluate the appropriateness of a potential site of care shift.

Q10: What if providers say their treatment plan will not support this change?

A10: CVS Specialty is committed to supporting the patient-provider relationship and avoiding any disruption or change to providers' treatment plans wherever possible. In some cases, however, changes to treatment plans may be necessary to meet authorization or formulary requirements. In those cases, the Specialty Pharmacy staff will work with members and their provider(s) to determine how we can best support the treatment plans to help ensure disruption is minimized and all clinical needs are met. In select cases where there is an extenuating clinical situation or a network gap, UPS may review, evaluate and approve a benefit exception.